



COMPLAINT SUBMISSION CHECKLIST

I, confirm that I have complied with the complaint submission requirements. I understand that the complaint will not be registered if the necessary details have been omitted.

(Please indicate the appropriate box)

Have you completed the beneficiary/member details, including contact details?	
If submitting complaint on behalf of beneficiary, did you obtain signed consent letter?	
Have you provided a detailed summary of the complaint and issues in dispute?	X
Did you attach proof of escalation to the medical scheme / other regulated entity?	
Please mark the supporting documents attached (MS Word or PDF.)	
Short paid / Unpaid account statements	
Clinical reports from treating doctors	
Radiology reports (X-ray; MRI; CT scan; Ultrasound etc.)	
Pathology reports (Blood test results; histology reports etc.)	
Clinical Motivation	
Email correspondence	X



Email: complaints@medicalschemes.co.za

Call Centre: 0861 123 267

Complaints Adjudication Unit
Member Protection Division
Council for Medical Schemes

COMPLAINT FORM

COMPLAINT LODGED IN TERMS OF SECTION 47(1) OF THE MEDICAL SCHEMES ACT, 131 OF 1998

Member / Beneficiary Details* (Please refer to the Notes at the bottom of the form¹):

Name(s)			
Surname			
ID number			
Email address			
Preferred contact no.		Alternative no.	
Postal address			

Complainant*: (applicable if complaint is submitted on behalf of the member or registered dependant)

Name(s)	Wilbur William		
Surname	Matthee		
ID number	8909035151087		
Email address	info@ethichawks.co.za		
Preferred contact no.	0633788782	Alternative no.	0696351435
Postal address	None/destitute as a result of employer Medscheme conduct		

*Mandatory fields - NB: Please complete all required fields to avoid a delay in registration of your complaint.

¹ Notes

- If mandatory fields are not completed, the complaint will not be registered.
- ID Number of principal member and affected beneficiary are mandatory for verification of identity and medical scheme membership.
- Kindly note that confidential and / or medical information will be communicated to the e-mail address provided. Please ensure that the e-mail address provided is correct. The CMS will not accept responsibility if the email address provided turns out to be incorrect.
- Postal address is optional only where a valid email address is provided.
- A complaint submitted by a third party on behalf of the member / beneficiary, will only be registered if accompanied by a signed consent letter authorizing the Registrar to correspond with the third party and to share personal / medical information with regulated entities for purposes of resolving the dispute. The consent letter must include contact details of the member/beneficiary for verification purposes.

Complaint lodged against:

Medical Scheme	Government Employee Medical Scheme
Member Number	000655789:02
Benefit Option	

Broker / Brokerage	
Broker Number	

Administrator	Medscheme Holdings
----------------------	--------------------

Managed Care Organization	Active disease risk management
----------------------------------	--------------------------------

1. Facts of the complaint (Please attach supporting documents/evidence² – Refer to Note 2 below)

The complainant, SP Mahlangu (Member Number: 000655789), is acting on behalf of his 75-year-old mother, his dependent. The core complaint is that the medical scheme incorrectly sent his mother an email accusing her of not claiming her chronic medication, causing her distress.

Key Facts:

- The Trigger: The dependent received an automated email on 19 October 2025 at 13:28 from NOCGEMSBHRM@MEDSCHEME.CO.ZA stating that regular monthly claims for chronic medication were not being received.
- Member's Position: The complainant states his mother is compliant and collects her cholesterol medication every month. He demands the scheme corrects its records and stops confusing her.
- Root Cause Identified: An investigation by Case Manager Wilbur Matthee determined the issue was a system integration error.
 - The dependent claimed medication from the acute benefit until 17 December 2024.
 - From January 2025, the medication was correctly claimed under the chronic benefit.
 - The DMS (Disease Management System) did not receive the update from Gexus (the claims system) about this benefit change. Therefore, DMS continued to monitor adherence based only on acute claims, found none, and automatically flagged the member as non-adherent.
 - This error triggered automated non-adherence letters on 19 May 2025 and 19 Oct 2025, despite the member being fully compliant under the chronic benefit.

² Note 2

- a) Copies of account statements, Doctor / Specialist reports, pathology reports, radiology reports (X-ray; MRI; CT scan) and any other documents relevant to the issues in dispute.
- b) A complaint will not be registered unless the necessary supporting documents mentioned above are attached.
- c) Supporting documents must be in MS Word or PDF format. Other file types must first be converted to PDF before submitting.

- Conflicting Internal Views: Team Leader Mary-Ann Jones initially insisted the non-adherence flags were correct based on a "twice in a rolling three months" methodology applied to the general claims history, overlooking the proven system error related to the benefit change.

Instructions by the Team Leader not to call the daughter who is complaining, but the call the 75 year old member and probe gently on adherence(possibility causing more harm than good). My Ethical refusal to call the member with misleading information is later referred to as insubordination in a recorded transcript.

Summary of Violations:

Mary-Ann Jones, Team Leader at GEMS ADRM, demonstrated gross negligence and unethical conduct by:

1. Ignoring Conclusive Evidence of a system error (Gexus-DMS integration gap) confirmed by investigation.
2. Insisting on Misleading Compliance Action against a member known to be compliant, demanding staff question an elderly patient based on false data.
3. Abusing Authority by labeling a subordinate's ethical refusal to propagate error as "insubordination," suppressing correction and endangering patient trust.

This poses a severe P1 risk:

- Patient harm via distress and potential mismanagement.
- Erosion of data integrity and clinical oversight.
- Culture of negligence where procedure overrides truth.

Required Immediate Actions:

1. Suspend Mary-Ann Jones from all supervisory/clinical duties pending disciplinary hearing.
2. Retract her incorrect guidance and correct the member's record.
3. Mandate retraining in ethics, root cause analysis, and professional accountability.
4. Formally acknowledge the investigator's correct findings and protect staff who report system errors.

Justification:

Healthcare administrators must uphold data integrity and patient safety. Her actions violate these duties, warranting swift intervention by the Council to uphold scheme integrity and member protection.

2. Dispute resolution steps undertaken (NB: Please attach proof of escalation to the entity complained against).

Step 1: Initial Escalation & Investigation

- The complaint was received and escalated internally to the GEMS Active Disease Risk Management (ADRM) team.
- Case Manager Wilbur Matthee investigated (Email dated 23 October 2025, 12:40 PM).
- He identified the root cause (Gexus-DMS miscommunication) and attached evidence.
- Proof of Escalation: Email from Wilbur Matthee to Team Leader Mary-Ann Jones (Page 2).

Step 2: Internal Disagreement & Refusal to Escalate Erroneously

- Wilbur Matthee, based on his findings, declined to contact the member as originally instructed, stating “Given that the integration gap still exists, I will not contact the member at this time.” He recommended a system review instead.
- This act was later characterized by Mary-Ann Jones in her brief response (Page 1: “I am not in agreement with your feedback though”) and in the user’s note as “insubordination,” indicating an internal dispute over the correct resolution path.
- Proof of Escalation/Dispute: The email chain itself shows the unresolved disagreement between the investigator (Matthee) and the team leader (Jones) on how to proceed.

NB: Include call reference numbers, emails sent and/or received from the medical scheme / entity)

3. Recourse required.

-
1. Correct the Record: The member’s adherence record must be updated to reflect her true compliant status, removing the incorrect non-adherent flag.
 2. System Correction: The underlying integration gap between the Gexus and DMS systems must be fixed to prevent this error from recurring for this member and others.
 3. Direct Communication & Apology: A formal apology should be provided to the member and her daughter for the distress caused by the incorrect and misleading communication. This should acknowledge the system error as the cause.
 4. Assurance: Provide assurance that the member’s benefits and records will not be negatively impacted due to this administrative error.

Proposed Resolution Text (as suggested internally on Page 6):

"We have identified that the communication you received was the result of a system error... Please be assured that your adherence status will be updated correctly going forward, and this will not affect your record. We sincerely apologize for any inconvenience caused..."

WW. Matthee

SIGNATURE

20th January 2026

DATE